Paranoid Personality Disorder (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

Paranoid personality disorder (PPD) is a long-standing pattern of distrust and suspicion in which other people's motives are read as harmful, beginning by early adulthood and showing up across many situations (American Psychiatric Association [APA], 2022). It belongs to **Cluster A** (odd or eccentric) with schizoid and schizotypal personality disorders. Unlike delusional disorder or schizophrenia, the suspicion is not held with psychotic certainty.

How common: in a large U.S. national survey, 4.41% of adults met criteria for PPD, making it one of the more common personality disorders (Grant et al., 2004). Rates vary widely depending on how studies assess personality disorders (Volkert et al., 2018).

A neglected disorder. PPD is often confused with other diagnoses and met with pessimism about treatment; it receives little clinical attention compared with its frequency (Carroll, 2009; Lee, 2017).

Counselor framing: these clients rarely come asking for help with trust. They come for depression, anxiety, conflict at work, or marriage problems, and they watch the counselor closely for any sign of betrayal.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis. A pervasive distrust and suspiciousness, beginning by early adulthood, with **4 or more** of:

- Suspects, without enough basis, that others are **exploiting, harming, or deceiving** him or her.
- Preoccupied with unjustified **doubts about the loyalty or trustworthiness** of friends or associates.
- **Reluctant to confide** in others for fear the information will be used against him or her.
- Reads **hidden demeaning or threatening meanings** into harmless remarks or events.
- Persistently **bears grudges** and is unforgiving of insults, injuries, or slights.
- Perceives **attacks on character or reputation** that others do not see, and is quick to react angrily or counterattack.
- Has recurrent, unjustified **suspicions about the faithfulness** of a spouse or partner.

Rule out: the pattern must not occur only during schizophrenia, a mood disorder with psychotic features, or another psychotic disorder, and must not be due to a medical condition or substance use. Also consider realistic mistrust shaped by past mistreatment, discrimination, or a dangerous environment (APA, 2022).

When counselors must refer (general clinical consensus): refer for psychiatric evaluation if suspicions become fixed and unshakable (possible delusions), if there are threats of violence or plans for revenge, or if substance use is heavy. Assess risk to others, including partners, with care (Carroll, 2009).

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in paranoid personality disorder. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
GPTS (Green Paranoid Thought Scales)	Ideas of reference and persecution (suspiciousness).	Self-report, 32 items	No (free)	None beyond general clinical training	Original article
SCID-5-PD (Structured Clinical Interview for DSM-5 Personality Disorders)	Diagnosing all ten DSM-5 personality disorders, with an optional self-report screener.	Semistructured clinician interview	Yes	★ Clinical interviewing skill; training recommended	APA Publishing

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
PID-5 (Personality Inventory for DSM-5)	Dimensional personality trait profile (negative affect, detachment, antagonism, disinhibition, psychoticism); brief and informant forms.	Self-report; 220-item full or 25-item brief form	No (free from the APA for clinical use)	None beyond general clinical training	APA assessment measures
PAI (Personality Assessment Inventory)	Broad clinical and personality assessment for adults, including scales for borderline and antisocial features.	Self-report, 344 items, about 50 to 60 minutes	Yes	★ Level C	PAR
MCMI-IV (Millon Clinical Multiaxial Inventory-IV)	Adults in treatment; personality patterns alongside clinical syndromes.	Self-report, 195 true/false items, about 25 to 30 minutes	Yes	★ Level C	Pearson
MMPI-3 (Minnesota Multiphasic Personality Inventory-3)	Broad personality and psychopathology assessment, with validity scales for over- and under-reporting.	Self-report, 335 true/false items, about 25 to 50 minutes	Yes	★ Level C	Pearson

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: treatment and the evidence

Bottom line for class: there are almost no treatment trials for PPD alone. Treatment rests on a patient, respectful alliance and on cognitive and schema-focused approaches studied across personality disorders.

Evidence overview (honest effect sizes):

- **Schema therapy:** in a multicenter trial of 323 patients with avoidant, dependent, obsessive-compulsive, paranoid, histrionic, or narcissistic personality disorder, schema therapy (which targets long-standing core beliefs about self and others) led to recovery more often than usual care or another psychotherapy (Bamelis et al., 2014). Few participants had PPD, so results for PPD specifically are uncertain.
- **No PPD-specific trials:** reviews describe the treatment evidence as very limited and rely on clinical guidance (Carroll, 2009; Lee, 2017).
- **Medication (referral notes):** no medication is approved for PPD. Medication may treat co-occurring depression, anxiety, or brief psychotic symptoms; these clients may be suspicious of medicines too, so clear explanation matters.

Cognitive work with PPD: how it is used (Bamelis et al., 2014; Carroll, 2009):

- **Build trust slowly.** Be transparent, predictable, and honest about notes, confidentiality, and limits. Avoid pushing for disclosure too early or confronting suspicions head-on.
- **Identify and challenge core beliefs:** "People are out to get me," "If I let my guard down, I'll be hurt," "Any kindness hides a motive," "Forgiving means letting them win."
- **Test interpretations:** list other explanations for an ambiguous event, run small behavioral experiments in trusting, and track what actually happens.

5. Religion, spirituality, and Christian integration: what the evidence shows

Disorder-specific research. We found **no study of religion or spirituality in paranoid personality disorder** and no faith-integrated treatment trial (literature search, November 1, 2026).

Related evidence. Across 49 studies, positive religious coping (trusting God, seeking His help) went with better adjustment to stress, and negative religious coping (feeling punished or abandoned by God) with worse (Ano & Vasconcelles, 2005). Across 97 outcome studies, therapy adapted to clients' religious beliefs worked about as well as standard therapy for distress and produced added spiritual benefits (Captari et al., 2018).

Honest summary for teaching:

1. A trusting alliance and cognitive, schema-focused work are the best available approach. Faith integration supports them; it does not replace them.

2. There is no research on faith and PPD. Draw on general evidence about religious coping and religiously adapted therapy.
3. Suspicion can extend to churches, pastors, and God. Never pressure trust; earn it.

6. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within cognitive, alliance-centered care. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for spiritual forms of mistrust: "The people at church are talking about me," "God lets people hurt me," "If I forgive, they'll do it again," "I can't trust anyone, not even God" (Ano & Vasconcelles, 2005).
- **Challenge (clinical).** Separate wise caution from blanket suspicion. Weigh the evidence for each specific fear, and practice small, safe risks of trust with people who have proven reliable (Bamelis et al., 2014). **Possible cognitive errors** to watch for, with their definitions from the Belief Counseling Cognitive Errors Checklist and the symptoms they tend to feed:
  - **Mind reading** — assuming I know what others think. *Feeds:* suspicion. *Sounds like:* "They're talking about me."
  - **Jumping to conclusions** — deciding without the facts. *Feeds:* grudges and hostility. *Sounds like:* "He forgot to call, so he's against me."
  - **Confirmatory bias** — looking for proof I'm right, ignoring the rest. *Feeds:* a growing sense of threat. *Sounds like:* "See? They're against me again."
  - **Selective attention** — noticing only what fits my belief. *Feeds:* reading hidden insults into remarks. *Sounds like:* "I noticed how she said "good morning.""
  - **Overgeneralization** — one event becomes a rule. *Feeds:* mistrust of everyone. *Sounds like:* "I can't trust anyone."
  - **Fortune telling** — predicting a bad outcome. *Feeds:* reluctance to confide. *Sounds like:* "If I tell her, she'll use it against me."
- **Questions to dispute the belief.** Have the client choose questions from the Questioning False Beliefs form that "speak to" the belief. Selected questions that fit the errors above:
  - "What's the actual evidence for my belief?" *For:* mind reading.
  - "Are there other ways of looking at this situation?" and "How might a good friend view this situation?" *For:* jumping to conclusions.
  - "What are some of the best arguments against my belief or conclusion?" *For:* confirmatory bias.
  - "Is it possible that I am distorting the facts?" *For:* selective attention.
  - "How might an all-loving God look at me or this situation?" *For:* overgeneralization and "I can't trust anyone, not even God".
  - "If I were a betting person, would I bet a million dollars that the thing I dread will happen?" *For:* fortune telling.
- **Replace (pastoral).** Offer Scripture of God's protection and of forgiveness that releases bitterness without denying wrong:
  - "When I am afraid, I put my trust in You. In God, whose word I praise—in God I trust; I will not be afraid. What can man do to me?" (Psalm 56:3–4)
  - "I will lie down and sleep in peace, for You alone, O LORD, make me dwell in safety." (Psalm 4:8). Security that does not depend on constant watchfulness.
  - "Love is patient, love is kind… it is not easily angered, it keeps no account of wrongs." (1 Corinthians 13:4–5). Letting go of the grudge ledger.
  - "Get rid of all bitterness, rage and anger, clamor and slander, along with every form of malice. Be kind and tenderhearted to one another, forgiving each other just as God in Christ forgave you." (Ephesians 4:31–32). Forgiveness is not the same as trusting an unsafe person.
- **Faith community (clinical).** Encourage one or two steady, discreet relationships at church rather than large groups. Gossip, broken confidences, or sudden changes can confirm the client's worst fears.
- **Ministers:** be consistent, keep confidences, avoid arguing over every suspicion, and refer when suspicion becomes fixed belief or when anyone may be at risk.

7. Quick classroom takeaways

- PPD is long-standing, pervasive distrust (4 of 7 criteria) without psychotic certainty. It affects about 4% of U.S. adults but is rarely the reason clients seek help.

- No PPD-specific trials exist; schema and cognitive therapy across personality disorders, built on a careful alliance, are the best evidence.
- Replace "people are out to get me" with the truth that God is a refuge, and with forgiveness that frees without requiring blind trust.
- Confirm with a structured interview (SCID-5-PD); the PAI, MCMI-IV, and MMPI-3 add detail but require Level C qualification.

Sources note

All 8 references were checked against Crossref, OpenAlex, and PubMed/Europe PMC records (Nov 1, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

References

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders (5th ed., text rev.)*. https://doi.org/10.1176/appi.books.9780890425787

Ano, G. G., & Vasconcelles, E. B. (2005). Religious coping and psychological adjustment to stress: A meta-analysis. *Journal of Clinical Psychology*, *61*(4), 461–480. https://doi.org/10.1002/jclp.20049

Bamelis, L. L. M., Evers, S. M. A. A., Spinhoven, P., & Arntz, A. (2014). Results of a multicenter randomized controlled trial of the clinical effectiveness of schema therapy for personality disorders. *American Journal of Psychiatry*, *171*(3), 305–322. https://doi.org/10.1176/appi.ajp.2013.12040518

Captari, L. E., Hook, J. N., Hoyt, W. T., Davis, D. E., McElroy-Heltzel, S. E., & Worthington, E. L. (2018). Integrating clients' religion and spirituality within psychotherapy: A comprehensive meta-analysis. *Journal of Clinical Psychology*, *74*(11), 1938–1951. https://doi.org/10.1002/jclp.22681

Carroll, A. (2009). Are you looking at me? Understanding and managing paranoid personality disorder. *Advances in Psychiatric Treatment*, *15*(1), 40–48. https://doi.org/10.1192/apt.bp.107.005421

Grant, B. F., Hasin, D. S., Stinson, F. S., Dawson, D. A., Chou, S. P., Ruan, W. J., & Pickering, R. P. (2004). Prevalence, correlates, and disability of personality disorders in the United States: Results from the National Epidemiologic Survey on Alcohol and Related Conditions. *The Journal of Clinical Psychiatry*, *65*(7), 948–958. https://doi.org/10.4088/JCP.v65n0711

Lee, R. (2017). Mistrustful and misunderstood: A review of paranoid personality disorder. *Current Behavioral Neuroscience Reports*, *4*(2), 151–165. https://doi.org/10.1007/s40473-017-0116-7

Volkert, J., Gablonski, T.-C., & Rabung, S. (2018). Prevalence of personality disorders in the general adult population in Western countries: Systematic review and meta-analysis. *The British Journal of Psychiatry*, *213*(6), 709–715. https://doi.org/10.1192/bjp.2018.202

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